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HAVEN PAEDIATRIC CENTRE

The Operations Handbook

Running a children's hospital, from the inside

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PART ONE. THE JOB

01 What the job actually is

An operations lead converts intention into reliability. The hospital already intends to give excellent care, and on most days it does. Your job is to make the good day repeatable when the senior nurse is on leave, the power has gone twice, two admissions arrive together and nobody has slept. Reliability is the product. Everything in this handbook is a way of manufacturing it.

There is a distinction that separates a junior operator from a senior one, and it is worth learning before anything else, because you will meet it a dozen times a week. A junior operator does the job. A senior one builds the thing that does the job.

Take a stockout. The ward needs a drug and the shelf is empty. A junior operator finds the drug, which is admirable and urgent and solves the problem for one patient on one afternoon. A senior operator finds the drug, and then asks why the reorder point did not fire, discovers there is no reorder point, and spends the following Tuesday setting reorder points for the forty items that account for most of the pharmacy's movement. The second person has had a worse week and a better year.

> You will be judged in your first month by how many fires you put out, and in your first year by how few there were to put out. Only the second measure is real.

You are also, and this is less obvious, the only person in the building whose job is the whole. The neonatologist owns the babies. The matron owns the nursing. The accountant owns the ledger. Every one of them is responsible for a part, and each part can be run perfectly while the hospital as a system performs badly, because the failures live in the joins. The patient who waits ninety minutes between being seen and being dispensed to is not anybody's failure. It is yours, because the join is yours.

What you are not

You are not the most senior clinician and you are not a deputy medical director. Section 15 draws that boundary properly and you should treat it as the hardest line in this handbook. You are also not a fixer whose value is personal heroics. If the hospital runs well because you are in it and badly when you are away, you have built nothing.

02 The numbers you must know by heart

A number you have to look up is a number you do not manage. I will ask you for these, without warning, and the right answer is the level and the direction, not the decimal place. "NICU occupancy is a bit over half and it has been climbing for three weeks" is a correct answer. "I will get back to you" is not.

TIED UP
N7M
N4.2m in HMO receivables, N2.77m in pharmacy stock

OF THE TARIFF
80.9%
1,952 of 2,413 lines priced at zero

BELOW MARKET
34%
on the prices that do exist

The ones that matter here

The number	Where Haven stands	Why it matters
Beds and cots	5 paediatric, 3 NICU	Your capacity ceiling. Everything is per bed-day.
Occupancy	Low, especially NICU	The NICU is the highest-yield asset in the building and it is not full.
Revenue a month	Roughly N15 to 20 million	The baseline the whole plan moves from.
Revenue per NICU admission	Around N3 million	Three cots make this the largest single lever in the business.
Average length of stay	Know it per unit	Turnover times yield is the real revenue equation, not yield alone.
HMO receivable	N4.2 million, mostly Leadway and NEM	Close to a full period's revenue sitting in two ledgers.
Debtor days	Compute it monthly	The honest measure of whether billing works.
Pharmacy stock	N2.77 million	Cash on a shelf. Moving to consignment frees it.
Stockouts a month	Count them	The number that tells you whether your reorder discipline is real.
Tariff lines priced	19.1 per cent	Four lines in five describe work that is done and never billed.
Payroll as a share of revenue	Track it	The dominant cost in a hospital this size, and the first thing a board asks about.
EBITDA margin	The floor is 18 to 20 per cent	What the board has committed to and will be measured on.

How to hold them

Keep one page. Write the number, last month's number, and the direction. Update it on the same day every month and never let somebody else update it for you, because the discipline of entering the figure yourself is how you notice when it is wrong. A figure that surprises you is the most useful thing that will happen all month, and you will only be surprised if you knew what to expect.

Your numbers, as at the end of your first week. Write them in pen.

03 The rhythm

Hospitals do not fail dramatically. They drift, and drift is cured by cadence. Three meetings run the operational year, and the discipline is not what happens in them but that they never move.

The daily huddle

Fifteen minutes, standing, same place, same time, every working morning. Standing matters because a seated meeting expands to fill the chairs.

> *Three questions, in this order, and nothing else.*
>
> *What happened overnight that the day shift needs to know?*
>
> *What is the plan for each patient in a bed today, and who is expected to go home?*
>
> *What will stop us today, and who is fixing it?*

The discharge question is the one people skip and the one that pays. A hospital that decides at nine in the morning who is going home has a free bed by noon. A hospital that decides at four in the afternoon has the same patient overnight, which costs a bed-day and gains nothing.

The weekly operating review

An hour, same day each week, with a fixed agenda so nobody prepares for a different meeting than the one that happens. Occupancy and admissions, the cash position and what moved on receivables, stock exceptions and stockouts, people and roster, the open actions from last week with names against them, and anything escalating. Actions carry an owner and a date or they are not actions, they are conversation.

The monthly pack

One page to the board. What moved, what did not, what you need decided. A board that reads one page every month is worth more to you than a board that is sent twenty pages and reads none. Put the decision you need at the top, not the bottom, because that is the part you actually need them awake for.

The temptation in a bad month is to write more, explaining. Resist it. The page gets shorter when things are difficult, not longer, and the explanation happens in the room.

PART TWO. THE MONEY

04 Profit is not cash

This is the most important idea in the handbook and the one most new operators arrive without. Haven is not short of money because it loses money. It is short of money because its money is somewhere other than the bank.

Work through it concretely. Suppose in a month the hospital bills N20 million. Of that, N8 million is self-pay and lands immediately. N12 million is on HMO terms and will be collected somewhere between forty and ninety days later, if the claims survive vetting. In the same month the pharmacy buys N3 million of stock, paid for now, which will be dispensed over the coming weeks. Salaries of N9 million go out on the twenty-fifth regardless.

The books will show a profitable month. The account will show N8 million in and N12 million out, and the person signing salaries will say, with complete honesty, that there is barely anything left after payroll. Both are true. The hospital is profitable and cash poor at the same time, and it will stay that way, structurally, until somebody changes the cycle rather than the margin.

> "There is nothing left after salaries" is a sentence about cash. Almost nobody who says it means it as a sentence about profit, and almost everybody who hears it treats it as one.

Why the two are cured differently

A profit problem is cured by price and cost. You charge more, or you spend less, or you change the mix towards the work that earns more. A cash problem is cured by the cycle: collect faster, hold less stock, pay later, bill sooner. Apply the wrong cure and you work hard at the wrong thing for a year. Cut costs in a hospital that has a collection problem and you will damage care without releasing a naira.

Haven has both, but the cash problem is larger, faster to fix and almost entirely within your control. That is unusual and you should be glad of it.

The cycle, in the only terms you need

Money leaves Haven when it buys stock and pays people. It returns when a patient or an insurer pays. The gap between those two moments, multiplied by how much is moving, is how much cash the hospital must have standing still at all times. Shorten the gap and cash appears without a single extra patient.

Three levers shorten it here. Collect the N4.2 million sitting in HMO ledgers, and then keep collecting on a monthly discipline so it does not rebuild. Move the pharmacy to consignment, so the supplier owns the stock on Haven's shelf and Haven pays on dispense, which converts N2.77 million of standing cash into somebody else's problem and ends the expiry risk at the same time. And bill at the point of care rather than days later, because a claim submitted late is a claim rejected on a technicality.

Debtor days, and how to compute it

Take the receivable balance, divide by the revenue for the period, multiply by the number of days in the period. If Haven is owed N4.2 million against N18 million of monthly revenue, that is roughly seventy days of sales sitting outstanding. Watch the trend rather than the absolute, and watch it by payer, because an average hides the one insurer who is the actual problem.

05 The tariff

Haven's tariff runs to 2,413 lines and 80.9 per cent of them carry a price of zero. Nineteen hundred and fifty two lines describe something this hospital does, and charge nothing for it.

That is not fraud and it is not laziness. It is what happens when a clinical team builds a service catalogue to record what they do, nobody is accountable for pricing it, and the list grows faster than anyone's attention. New services arrive and are added for the record. Templates are copied in with their price columns empty. Two years of that and you have Haven's tariff.

Why this is the fastest money in the building

Repricing requires no equipment, no hire and no building. It requires somebody to sit down with a list and do the work. Pricing the unpriced lines and closing the gap to market is worth at least N2 million a month before a single additional patient walks through the door, which is a greater immediate gain than almost anything else in the strategy and the only one you could in principle finish by Christmas.

How to do it without inventing numbers

Every price has to be defensible, because you will have to defend it, to a parent at a desk and to an HMO on a call. Benchmark against a real peer hospital's schedule and against the Lagos market, line by line, and keep the source. A price you can source is a price you can hold. A price you invented is one you will discount the first time somebody pushes.

Raise in two tranches across the year rather than one jump. A 45 per cent correction delivered in a single morning reads to a family as opportunism, however justified the arithmetic. The same correction in two steps, with the service visibly improving between them, reads as a hospital growing up.

Anchor on self-pay first and set the HMO schedules separately and lower. They are different businesses with different economics and treating them as one schedule is how hospitals end up losing money on their busiest payer.

The part everybody forgets

A price that exists in a spreadsheet and not in the billing system is not a price. The repricing work is only half done when the schedule is agreed. It is done when a nurse records a procedure and the charge appears on the bill without anybody remembering to add it. Until then you have an expensive document.

For each line, four questions	
Do we actually do this?	If not, retire the line. A shorter true tariff beats a longer aspirational one.
What does a comparable Lagos hospital charge?	This is your anchor and your defence.
Is this self-pay, HMO, or both, and at what rates?	Two schedules, set separately.
When this is done, does it reach the bill automatically?	If the answer is "if someone remembers", it is not finished.

Work at a fixed rate. Twenty lines a day, every day, and the whole tariff is done inside a quarter without ever being the only thing you did. Start with the lines that move most often rather than the top of the alphabet.

06 Getting paid

The payer mix is a decision, not a fact

Self-pay and HMO are two different businesses. HMOs fill beds, which matters when beds are empty, but they pay a fraction of self-pay rates, they pay late, and they will not cover a NICU stay at anything like its cost. Self-pay is the anchor. HMO volume is balanced against margin deliberately, not chased because volume feels like progress.

Know your mix by revenue and by bed-day, and know it per unit. A hospital can be busy, insured and quietly unprofitable, and the only thing that reveals it is the mix.

Where the HMO cycle breaks

The chain runs: authorisation, encounter, documentation, claim submission, the insurer's vetting, payment, reconciliation. It breaks in four predictable places, and all four are yours.

Break	What it looks like	The fix
No authorisation	Care given in good faith, claim refused outright	Authorisation before the encounter, no exceptions, and a named person who chases codes
Thin documentation	Claim queried, then aged, then written off	The note supports the claim at the time of care, not reconstructed later
Late submission	Rejected on a technicality that has nothing to do with the care	A submission deadline measured in days, tracked weekly
No reconciliation	Nobody can say what was paid against what was billed	A monthly line-by-line match, with a named owner

The first fortnight

Your early win here is the two largest balances. Leadway and NEM together hold most of the N4.2 million. Do not send a letter. Get the reconciliation done first so that you arrive at the call knowing their number and your number and exactly where the two diverge, then ring the person rather than the organisation.

> *I have reconciled our account to the line. We show N__ outstanding across __ claims from __ to __. Your statement shows N__. The difference is __ claims, and I have the documentation for each of them ready to send today. Can we agree the undisputed portion now and set a date for the rest?*

The sentence that moves money is "I have reconciled to the line." Most providers calling an insurer have not, and the insurer knows it within thirty seconds.

After that, the discipline is monthly and dull, and the dullness is the point. Receivables rebuild quietly the moment the discipline lapses.

07 Reading the accounts without being an accountant

You do not need to be an accountant. You do need to be impossible to mislead, including by yourself.

Markup is not margin

Haven's reporting has described a pharmacy profit of 201 per cent. That is a markup, not a margin, and the two are different enough to change decisions.

Buy an item for N100 and sell it for N301. The markup is 201 per cent, because you added 201 on top of 100. The margin is 201 divided by 301, which is 66.8 per cent, because margin is measured against what you sold it for. Both describe the same transaction. Only the second one can be compared with anything else in the business, and only the second one tells you what share of the revenue you actually keep.

Whenever somebody quotes you a percentage, ask what it is a percentage of. It is the cheapest question in management and it will save you from most bad decisions.

The reconciliation that matters most

Three counts should agree: how many patients were seen, how many were billed, how many were receipted. At Haven they do not. When counts disagree, the smallest is the closest to the truth, and the gap between the smallest and the largest is the size of your problem.

Do this monthly, by unit, and write down the gap. Watching that gap close is the single clearest evidence that your capture work is real, and it is the number I would put at the top of the board page for your first six months.

Two rules

Never present a number you cannot source. Not because somebody will catch you, though they will, but because an unsourced number cannot be acted on and so is worse than no number at all.

Say "I do not know yet" out loud and often. In a hospital whose record does not currently reconcile, the person who says it is the most trustworthy person in the room.

PART THREE. RUNNING THE HOSPITAL

08 Beds, cots and the NICU

The arithmetic of capacity

Five paediatric beds and three NICU cots. In a thirty-day month that is 150 paediatric bed-days and 90 cot-days available, and every operational decision you make either converts one of those into revenue or wastes it. Occupancy is bed-days used divided by bed-days available, and it is the most honest single measure of whether the hospital is working.

Revenue is not yield alone. It is yield multiplied by turnover. Two hospitals with identical prices and identical beds will earn very differently depending on how quickly a bed becomes available again, which is why discharge discipline is a revenue activity and not an administrative one.

Why the NICU is different

At roughly N3 million per admission across three cots, the NICU is the largest single lever in the business. It is also the one you cannot fill by working harder at the front desk, because Haven does not deliver babies. Every baby in those cots was referred in by somebody who chose Haven.

That makes the referral relationship the supply line for your most valuable asset. Obstetricians and maternity units keep their own delivery and antenatal revenue; what Haven earns is the baby who needs neonatal care they cannot give. Said that way it is not competition, and it is the only way it will ever be heard.

Operationally your part is readiness and confidence. A cot that is clean, equipped, staffed and available within the hour is what converts a relationship into an admission. A referring obstetrician who is told to wait twice does not call a third time.

Discharge, properly

Decide at the morning huddle who is going home today and what has to happen for it: the review, the prescription, the discharge summary, the follow-up appointment, the account settled. Assign each to a person at the huddle rather than hoping. The target is the patient leaving before noon, which frees the bed for the same day rather than the next one.

A discharge that slips to the evening costs a bed-day. In the NICU a slipped discharge costs a cot-day that a referring hospital was told was unavailable.

09 The roster and the establishment

Establishment is not headcount

Your establishment is the number of posts the hospital needs. Your headcount is the number of people you have. Confusing them is how hospitals end up permanently short while believing they are fully staffed.

Work it from the care, not from the budget. Decide the nursing hours a patient needs per day at each level of care, multiply by the beds you intend to run, cover every hour of every day including nights and weekends, then add an uplift for annual leave, sickness, training and turnover. Without that uplift the roster only works in a month where nobody is ill, which is no month.

Then look at skill mix. A rota that is full of bodies and short of the one person qualified to do a specific thing is not covered, and the NICU is where this bites hardest.

Roster rules that are worth defending

Publish four weeks ahead. People have children and lives, and a hospital that tells its staff their shifts on Friday for Monday will lose good people to hospitals that do not, whatever it pays.

Treat persistent overtime as a measurement rather than a cost. Overtime concentrated in one unit is telling you the establishment there is wrong. Paying it month after month without changing the establishment is paying a premium to avoid hearing the message.

Swaps go through one person and get recorded. A swap that lives in a WhatsApp message is a gap waiting for the week somebody forgets.

10 Stock, pharmacy and procurement

N2.77 million of Haven's cash is standing on shelves.

Consignment, and what it does not fix

Under vendor-managed inventory the supplier owns the stock sitting in Haven's pharmacy. Haven pays when an item is dispensed, and the supplier carries the expiry risk. That releases the cash and removes a category of loss that nobody ever budgets for.

What it does not fix is discipline. Consignment with no reorder points still produces stockouts, and consignment without counting still produces shrinkage. The supplier is carrying your capital, not your management.

The disciplines underneath

Sort the catalogue by movement. A small number of items account for most of the volume, and those are the ones that get reorder points, minimum and maximum levels, and your attention. The long tail gets ordered when needed and is not worth your Tuesday.

Set a reorder point that accounts for how long the supplier actually takes, not how long they promised. Count the fast movers weekly and everything else on a cycle, rather than shutting the pharmacy for a heroic annual count that is wrong by the following week.

Keep an expiry register and review near-expiry monthly, early enough to use the stock rather than discover it.

Match three documents before anything is paid: what was ordered, what was delivered, what was invoiced. Where those three are not matched by somebody other than the person who ordered, money leaves quietly and for years.

Controlled drugs stay under the pharmacist and the founders, with their own register and their own count. That one is not an efficiency question.

Stockouts are a number

Count them. One per month is a system working. Ten is a system that is not there, whatever anybody's effort. A stockout in a children's hospital is also a clinical event, which is why this section sits in operations and not in finance.

11 The front desk and the patient journey

The experience is the marketing. Haven already earns word of mouth, and a parent who felt seen tells every other parent they know, which is a more efficient acquisition channel than anything you could buy.

Map it before you change it

Walk the whole journey as a parent would, with a stopwatch and no announcement: arrival, parking, reception, registration, triage, the wait, the consultation, investigations, pharmacy, payment, departure, follow-up. Do it on a busy afternoon rather than a quiet morning. Write down every point where a parent has to ask somebody what is happening, because each one of those is a defect.

Where money leaks on this journey

Registration that captures incomplete payer details, so the claim fails weeks later. The item used and not recorded, which is the capture problem meeting the tariff problem. The co-payment not collected at the desk because it felt awkward, which it will feel much more awkward to collect in six weeks. The investigation ordered and never billed.

Most revenue leakage in a hospital this size is not theft. It is a hundred small omissions by people trying to be kind and busy, and it is cured by making the right thing automatic rather than by asking people to remember.

Where trust leaks

Waiting is tolerable. Waiting without information is not. A parent told "the doctor is with an emergency, it will be about forty minutes, can I get you water" waits more happily than a parent told nothing for fifteen. Door to doctor time is worth measuring, and worth publishing internally.

> *At the desk, when the wait has grown.*
>
> *I am sorry, the clinic is running about __ minutes behind because we have had an emergency in. I will come back to you at __ with an update either way. Can I get you anything in the meantime?*

The promise to come back at a stated time is the part that works, and it only works if it happens.

PART FOUR. THE PEOPLE

12 Standards of work

Haven's staff survey came back strong. Sixteen of seventeen rated the safety of care very good or excellent, anonymously, with every opportunity to say otherwise. That is the asset. Your job is not to improve the culture. It is to build the systems that let the culture survive doubling in size.

The difference between a procedure and a standard

A procedure says what to do. A standard of work says what good looks like, who checks it, and how often. A hospital full of procedures nobody checks has a filing cabinet. A hospital with twenty standards that are genuinely checked has a method.

Care should not depend on who is on shift. That sentence is the whole purpose of the exercise, and it is also how you should explain it to a nurse who has been doing it her way successfully for two years and does not see why she should change. You are not telling her she is wrong. You are making sure the patient gets her version when she is on leave.

How to write them

With the people who do the work, never at them. Start by watching and asking somebody to show you how they do it now, and write that down. Most of the time the existing practice is sound and undocumented, and the standard is a recording rather than a reform. Where it is not sound, you will find out why, and the reason is usually a constraint nobody told you about.

Keep each to a page. Date it, name an owner, put it where the work happens rather than in a folder, and review it on a schedule. An undated standard is a rumour.

Start with the handful where variation hurts most: admission, handover, discharge, drug administration, the crash trolley check, infection control, and the NICU routines. That is a quarter's work and it is the foundation everything else stands on.

13 Incentives

Haven's board already has a commission structure and a set of job descriptions and targets on its list. You will be asked to shape them, and this is the section where a green operator can do the most damage with the best intentions.

People do what you measure

Not what you say, and not what you intend. Measure activity and you will get activity, including activity that should not have happened. Measure revenue per clinician and you will get it, by routes you will not enjoy discovering.

The line you must not cross

Never pay a clinician for the volume of admissions, for length of stay, or for tests ordered. In a hospital the clinical decision must be insulated from the clinician's income, and a scheme that couples them is a patient safety risk before it is a commercial one, whatever the intention behind it. If somebody proposes it, and somebody will, that is a conversation for the Medical Director and the board, and your answer in the room is that you will not design it.

What you can reward is quality, reliability and ownership: standards met, documentation complete at the point of care, infection rates, readmissions, collection at the desk, stock discipline, patient-reported experience, attendance and reliability on the roster.

Make it understandable

If a nurse cannot explain in one sentence what she has to do to earn it, it will not change behaviour, it will only cost money. Simple and slightly imperfect beats elegant and opaque every time.

Recognition is cheaper than money and often stronger

Most of what makes people stay in a hospital like this is being seen. Name the person who caught the near miss. Say what they did and why it mattered, in front of their colleagues, within the week. That costs nothing, it is the most direct protection your survey result has, and it is unavailable to any competitor who is only willing to compete on salary.

14 The three conversations

The handover

Structured, same shape every time, so nothing depends on how articulate the person going off shift is at seven in the morning. Situation, background, assessment, recommendation. Who is sick, who is for discharge, what is pending, what you are worried about. Handover is where most hospital harm is prevented or created, and it is an operational artefact as much as a clinical one, because the structure is yours to install even though the content is theirs.

The huddle

Covered in section 3. The thing to protect is that it stays fifteen minutes and forward-looking. The moment it becomes a status report it will be forty minutes and people will stop coming.

The hard one

You will have to tell somebody their work is not good enough, probably within your first quarter, probably somebody who has been here longer than you.

Prepare. Know the specific instances, with dates. Have the standard you are measuring against, which is why section 12 comes first: it is very difficult to hold somebody to a standard that was never written down, and it is unfair to try.

In the room, be specific rather than general, describe behaviour rather than character, say what good looks like, listen properly to the reason because there often is one, agree what changes, and fix the date when you will both look again. Then write two lines of record and send them to the person the same day so you share one version of what was said.

> *I want to talk about the drug chart on Tuesday and the one on the ninth. On both, the administration was not signed at the time. The standard is that it is signed as it is given, because the next nurse has to be able to trust the chart. Help me understand what is making that difficult.*

The last sentence is not softness. Half the time the answer is that the chart is kept somewhere that makes signing at the bedside impossible, which is your problem rather than hers, and you have just saved yourself from disciplining somebody for your own system's defect.

Do it privately, do it early, and never in front of a parent.

PART FIVE. SAFETY AND GOVERNANCE

15 The clinical boundary

This is the hardest line in the handbook, and the one that will decide whether the clinical team trusts you.

What you own

Access, flow and capacity. Supply, so the right drug and the right consumable are there. The environment and the equipment, working and maintained. Staffing levels and skill mix. The record, as an operational and legal artefact. The standards of work, written with the clinicians. Everything that makes a good clinical decision possible to carry out.

What you do not own

The clinical decision itself. Not whether to admit, not whether to discharge, not what to prescribe, not what to write in the note. Dr Odedina and the medical team hold that and it does not become yours because you are senior, because the numbers are difficult, or because you are right.

Three things you must never do, whatever the pressure. Do not press for a discharge to free a bed. Do not ask anybody to change what is written in a record. Do not let a commercial target reach a clinician as a clinical instruction.

> The moment an operations lead is thought to be shaping clinical decisions for operational reasons, every subsequent conversation with that clinical team is a negotiation rather than a collaboration. You do not get it back.

When it is genuinely blurred

It often will be. A clinical decision with an operational consequence is still a clinical decision, and your route is to raise it with the Medical Director rather than act on it or argue at the bedside. You are allowed to say "that choice has this cost, I want you to be making it with that in front of you." You are not allowed to make the choice.

Being disciplined about this line is also what gives you standing when you need something clinical to change. The operations lead who has never once pushed into clinical territory is the one who is listened to when she says a routine is unsafe.

16 Incidents and near misses

Before anything else in this section: there is one historical matter at Haven that is governed by a standing rule, and it is set out in section 2 of your engagement guide. Read that before you write anything into a register, a report or a message about any incident at this hospital. If you are ever unsure whether a sentence falls under it, it does, and you call me.

Report to learn, not to blame

A hospital that investigates incidents to find a person gets fewer reports, not fewer incidents. The reporting culture is the asset and it is fragile, and a single investigation that is experienced as a hunt will cost you a year of reports.

Say what the register is for when you introduce it, and then prove it by how the first few are handled.

Near misses are the valuable ones

An incident is something that reached a patient. A near miss is something that was caught first. Near misses are free lessons and in most hospitals they are invisible, because nobody thinks to report the thing that did not happen.

Ask for them explicitly and thank people loudly for them. The month your near-miss reports go up is a good month, and you will need to explain that to a board whose instinct will be the opposite.

The discipline

A register with date, what happened, what was caught, the contributing factors, the action, an owner and a date. Reviewed on a fixed cadence with the clinical team. Actions closed out and visibly closed, because an action list that never closes teaches people that reporting changes nothing.

Look for the system cause rather than the person. Almost always you will find a standard that did not exist, a supply that was not there, a handover that did not happen, or a roster that left somebody doing two jobs. Those are yours.

17 The regulatory floor and the record

Nothing in this section grows revenue. All of it can stop the hospital, which makes it the cheapest insurance you will ever buy.

The licence register

One sheet, every licence and registration, with the expiry date and an owner, and a reminder ninety days ahead of each. Facility registration with the state health facility regulator. Every clinician's practising licence current with their own council, checked rather than assumed, because a lapsed licence is the hospital's exposure and not only the individual's. Premises, waste, and any laboratory or imaging permissions that apply.

Check this in your first fortnight. It is the single most common thing a new operator finds has quietly expired, and the only time it is cheap to discover is before somebody else does.

The record

A patient record is a clinical document, a legal document and the evidence behind every claim you submit. Those three purposes are served by the same discipline: written at the time, complete, legible, attributable, and kept for as long as the law requires.

Patient data is personal data under the Nigeria Data Protection Act. In practice that means collect what you need, keep it where it is controlled, let people see it only for a reason connected to their job, and know what you would do in the week you discovered a breach. Decide that in advance, because you will not invent a good answer under pressure.

What this looks like day to day

Audit a small sample of records every month against the standard, with the clinical team, as a learning exercise rather than an inspection. You will find the same three defects repeatedly, and fixing those three is worth more than a perfect policy nobody reads.

PART SIX. GROWTH

18 The ten levers, and the four that are yours

The strategy puts ten levers behind the move from roughly N18 million a month to the mid forties. You are not responsible for all ten. You are responsible for four of them and you make the other six possible.

Lever	Yours?
6. Reprice the tariff and fix capture	Yours. The fastest money and the clearest early win.
7. Free the working capital, consignment stock and receivables	Yours. Roughly N7 million, and it funds the rest.
2. The NICU filled and turning	Yours operationally. Dr Odedina owns it clinically.
8. Standards, roster, incentives, the culture that survives growth	Yours. The one that makes the others last.
1. Referral portal and a dedicated referral analyst	You enable. Someone else works the phones; you make the hospital worth referring to.
3. Paediatric day-case and elective surgery	You enable. Theatre readiness, lists, governance.
4. More HMO panels and corporate retainers	You enable. The empanelment paperwork and the payer mix discipline are yours.
5. The Haven Club membership	You enable. The promise is only as good as the operations behind it.
9. Home care and the add-on services	You enable. New services need standards before they need marketing.
10. Marketing and events	You enable. The experience is the marketing.

The pattern is worth seeing. Your four are the ones that need nobody's permission and no new capital. They are available to you in your first quarter, they are measurable, and they are what will establish whether the board was right to fund your seat.

The order

Tariff and capture first, because it is the fastest and because doing it teaches you the hospital's clinical activity more thoroughly than any briefing would. Working capital next, because it funds everything else. The NICU drive runs alongside both from day one, because it cannot wait. Standards and rhythm build underneath all of it and come good in the second quarter.

19 The high season

Paediatric demand in Lagos rises from November through February, and the entire six-month plan is built on reaching it ready. You arrive just before it.

Before it starts

Stock the fast movers to a level that assumes a bad week, not an average one. Build the roster for the season now, including the holidays, and get the leave plan agreed before people book flights. Make sure every cot is genuinely ready, equipped and staffed, rather than theoretically available. Have the referral conversations now, because an obstetrician decides where to send a baby based on a relationship that already exists, not on a call made in December.

During it

Do not start a system change in the middle of the season. A new process introduced in December will be blamed for everything that goes wrong in December, fairly or not, and you will lose the process and some credibility with it. Run what you built, protect the team, and keep a list of what you would change for next year.

The part people miss

The high season is when culture frays. Tired people are short with each other and with parents, and the survey result that is your best asset is most at risk in exactly the weeks you are busiest. Feed people. Notice them. Be visible at nights and weekends rather than only in office hours. The operations lead who is in the building at eight on a Sunday in January during her first season will be a different person to that team for the rest of her time there.

PART SEVEN. YOU

20 The first ninety days

Weeks one and two: understand

Change nothing structural. Walk every shift including nights, because the hospital at two in the morning is a different hospital and it is the one that tells the truth. Learn names, and learn who people actually go to when something goes wrong, which is often not who the organogram says.

Find out where the record actually lives rather than where it is supposed to live. In most young hospitals the real record is a notebook, a WhatsApp group or one person's memory, and you cannot fix a system whose true shape you have not found.

Check the licence register. Pull the last three months of the three counts in section 7 and see how far apart they are.

Weeks three to six: the tariff

Twenty lines a day. Benchmark, price, decide self-pay and HMO separately, and wire it into billing as you go rather than at the end. This is your first visible win and it should be visible: tell the board what the capture gap was when you arrived and what it is at the end of the month.

Weeks five to eight: the cash

Reconcile Leadway and NEM to the line, then ring them. Put the monthly receivables discipline in place so the balance does not rebuild. Start the consignment conversation with the pharmacy supplier.

Weeks seven to twelve: the rhythm

Install the daily huddle, the weekly operating review and the monthly board page. Write the first six standards of work with the people who do the work. By the end of the quarter the hospital should be able to see itself, which is the thing it has never been able to do.

Underneath all twelve weeks, the NICU readiness and referral work runs, because the season does not wait.

The thirty-day note

At the end of your first month write one page to the board: what you found, what you are doing first and why, what you need from them. Not a critique of your predecessors and not a list of everything that is wrong, which will be long and will make you no friends. Three things you are fixing, with dates.

The three things you will fix first, and the date each is done by.

21 Being new and senior at the same time

You will arrive with a plan. They will have a hospital. Both are legitimate and the second one has been keeping children alive for two years without you.

Earn the right to change things

The fastest way to be trusted is to fix something small that has annoyed people for months and that nobody senior has ever bothered with. A piece of equipment that has been broken since March. A rota that always breaks on the same day. The thing everyone complains about and no one owns. Fix that in your first fortnight and you will buy a year of goodwill for a week of effort.

Do not announce a transformation

Nobody who works here experiences their hospital as a problem to be solved. They experience it as a place they have held together. "Show me how you do this now" opens doors that "here is how we will do it" closes, and it also happens to be how you learn.

The person who knows most may be the most junior

A nurse two years in this building knows things that are in no document and will never be in one: which supplier actually delivers, which HMO code always gets queried, which machine fails in the heat. Ask her. Be seen to use what she tells you and to say where it came from.

You will be tested

Somebody will push, probably in a meeting, probably about something you do not yet know. The answer is not to bluff, because you will be found out and it is unrecoverable. "I do not know yet, I will have an answer by Thursday" and then having it by Thursday builds more authority than any amount of confident improvisation.

On being a doctor in an operations seat

Your clinical training will help you more than you expect, because you will understand what the clinical team is telling you and they will know it. It will also tempt you across the line in section 15, because you will often have a clinical opinion. Having it is fine. Acting on it from this seat is not.

22 When to ask for help

Call me about anything to do with money, anything involving the board or the owners' relationships, anything that changes what Haven is trying to be, and anything you are uncertain enough about that you would otherwise guess. Call Tito about the record, the audit findings, and what we actually saw in August rather than what the report summarised.

The standing rule is the one from the letter at the front of your guide. Tell me early, in the state the thing is actually in. Nobody has ever been in trouble with me for bringing bad news on a Tuesday.

There is one category where you should not wait at all, and that is anything you believe is unsafe. Not inefficient, not expensive, unsafe. That goes to the Medical Director immediately and to me the same day, and it does not wait for a meeting, a reconciliation or a better moment.

Last thing

You were brought into this because the hospital needs somebody whose job is the whole of it, and because I think you can learn faster than the season is going to move. Most of what is in this handbook you will only really understand the second time you meet it, in the building, with a decision in front of you. That is how operations is learned and there is no shortcut worth taking.

Mark it up. Argue with it. Bring me the parts you think are wrong.

Dr Debo Odulana

Founding Partner, Consult for Africa